

NHS Devon Integrated Care Board Service Specification

Service Specification No. :	V5– 10/07/26
Service:	Oral antipsychotics in primary care - interim care pathway
Commissioner Lead:	Warwick Heale, Director of Clinical Partnerships
Provider Lead:	
Period:	
Date of Review:	March 2027

1. Population Needs

National Context and Evidence Base

NICE guidance on the management of psychosis sets out clear expectations for the initiation and ongoing monitoring of oral antipsychotic medication. At the point of initiation, baseline physical health measurements should be undertaken within Secondary Care.

Responsibility for patient management typically remains with Secondary Care Services for a minimum of 12 months following initiation, or until the patient's condition has stabilised, whichever is longer. Following this period, and in line with the individual's care plan, ongoing monitoring may be transferred to Primary Care within agreed shared care arrangements.

Local Context

Locally, there is a significant cohort of patients prescribed oral antipsychotic medication within Primary Care without formal shared care arrangements in place:

- Approximately 11,000 patients are currently prescribed antipsychotics in Primary Care.
- Of these, around 8,000 patients are not actively managed within Secondary Care caseloads.

This reflects a substantial shift in clinical responsibility to Primary Care, without the corresponding infrastructure or formalised shared care frameworks typically expected for this medication group.

Following significant safety and medico-legal concerns raised by Devon Local Medical Committee (LMC) and GP Practices, development of formal shared care guidelines for oral antipsychotics has been identified as a system priority by NHS Devon ICB and mental health providers, to alleviate safety concerns in Primary Care and to close the historical commissioning gap for this patient group. However, establishing formal Shared Care agreements for all eligible patients currently prescribed antipsychotics by their GP requires significant additional resources in Secondary Care and will take time to implement.

Rationale for Service Development

In response to these pressures, the primary aim of this service is to support General Practitioners (GPs) in managing patients safely and effectively during the interim period prior to specialist review. The service seeks to ensure continuity of care, mitigate clinical risk, and provide appropriate recognition of the additional workload currently undertaken within Primary Care.

Scope:

- Adult patients prescribed oral antipsychotic medication who are clinically stable and who remain under Primary Care management while awaiting specialist review.

Exclusions:

- Children and young people (under 18 years)
- Patients prescribed antipsychotics under an existing formal shared care agreement, remunerated via the Specialised Medicines Service (SMS) LES
- Patients prescribed antipsychotics for behavioural and psychological symptoms of dementia (BPSD) – For advice on how to manage these patients, refer to Formulary guidance on Antipsychotics for severe agitation, aggression or psychosis in people with dementia
- Injectable antipsychotics
- Non mental health conditions e.g. palliative care, Huntington's disease, intractable hiccups etc.
- Patients who were initiated by a GP independently without this being requested by a specialist

2. Outcomes

NHS Outcomes Framework domains & Indicators

Domain 1	Preventing People from dying prematurely	✓
Domain 2	Enhancing quality of life for people with long term conditions	✓
Domain 3	Helping people to recover from episodes of ill health or following injury	✓
Domain 4	Ensuring people have a positive experience of care	✓
Domain 5	Treating and caring for people in a safe environment and protecting them from avoidable harm	✓

3. Scope

3.1 Aims and objectives of service

Recognising the time required for full implementation of specialist review and formal shared care agreements for all eligible patients, the primary aim of this service is to support General Practitioners (GPs) in managing patients safely and effectively during the interim period before specialist review.

This Local Enhanced Service (LES) provides funding to recognise and support the additional workload incurred by practices, including prescribing and patient monitoring responsibilities.

Under this service, while patients are awaiting specialist review/assessment, practices will continue prescribing and monitoring to ensure continuity of care and maintain patient safety.

Clinical judgement remains paramount in determining whether a patient is suitable to remain on this pathway. GPs should consider each patient's clinical stability and overall wellbeing on an ongoing basis.

If a patient's condition deteriorates or presents with escalating clinical concerns, an urgent referral to Secondary Care should be made in line with standard pathways and protocols.

3.2 Service description/care pathway

1. Patients currently prescribed oral antipsychotics in Primary Care who ARE on Secondary Care caseload and who are eligible for shared care will have a formal shared care request completed and sent to GP to request ongoing shared care prescribing and monitoring, with agreed funding via the Specialised Medicines Service (SMS) LES. In order to avoid an unmanageable peak in future annual reviews (see

paragraph 5, below), the first subsequent review for this cohort under the SMS LES might be extended, but there will still be specialist oversight as for any patients on secondary care caseload.

- a. If it is not clear that a patient would be eligible for shared care i.e. EUPD, the specialist will review this at the patient's next appointment (see point 4).
2. All patients currently prescribed oral antipsychotics in Primary Care (unless excluded – see below) who are NOT on Secondary care caseload will be referred to Secondary Care using a standard referral form that will enable Secondary care to prioritise patients for review.
3. For all patients pending shared care guidelines and / or specialist review:
 - a. Support is available to GP from mental health providers through care helpline and escalation processes
 - b. Whilst patients are in the “Oral antipsychotics in Primary Care - interim care pathway” General Practice should:
 - i. Undertake appropriate drug safety and physical health monitoring. If a patient is on high dose antipsychotics* they will have a higher priority for secondary care review and while awaiting that review, General Practice may opt to seek specialist advice on monitoring via the secondary care advice line. Once reviewed by secondary care, patients prescribed antipsychotics above licensed dose will not be eligible for shared care.
 - ii. Consider the following, where appropriate:
 1. providing the separately-funded SMI LES for eligible patients i.e. weight, BP, blood lipids, alcohol consumption etc.**
 2. reviewing the patient's smoking, alcohol and substance misuse status, with brief intervention and onward referral as appropriate
 3. Offering other support where appropriate and available e.g. ARRS Mental Health Practitioner input, social prescribing, signposting to alternative / additional sources of support e.g. VCSE organisations.
4. Following specialist review:
 - a. If suitable for shared care a formal shared care request will be completed and sent to GP to request ongoing shared care prescribing & monitoring, with agreed funding via the Specialised Medicines Service (SMS) LES.
 - b. If unsuitable for shared care, Secondary Care will consider deprescribing or retaining prescribing within specialist services.
 - c. For appropriate patients, Secondary Care may suggest deprescribing in Primary Care as an alternative to ongoing shared care, supported by clinical guidelines. Whilst this enhanced service does not direct GPs to deprescribe for patients it does provide for deprescribing where a doctor is competent to do so. GP Practices that agree to deprescribe will be able to continue to claim under this LES while deprescribing takes place.
5. Secondary Care providers will phase their reviews of referrals received under this LES in order to ensure an even distribution through the year so that future annual reviews can be managed in a timely manner. Practices will be able to claim retrospectively under this LES, rather than having to make a referral in order to claim payment (see section 7).
6. Patients who were initiated by a GP in the Practice independently without this being requested by a specialist should be considered for deprescribing in Primary Care. If the GP's clinical judgement is that due to patient risk there should be a Secondary Care review before deprescribing, the patient should be referred to Secondary care using the process noted above and the GP should continue prescribing in the meantime. Such patients will not be eligible for payment under this Oral antipsychotics in Primary Care - interim care pathway LES.

* High Dose Antipsychotic Therapy is defined by the Royal College of Psychiatrists as either:

7. A total daily dose of a single antipsychotic which exceeds the upper limit stated in the BNF or
8. A total daily dose of two or more antipsychotics which exceeds the BNF maximum as calculated by percentage. A cumulative dose of more than 100% is a high dose.

** If a patient is also eligible for monitoring under the SMI LES, there may be some overlap in some of the above interventions (i.e. cardiometabolic monitoring etc.), practices may wish to align this monitoring to avoid duplication and/or the need for additional appointments.

3.3 Population covered

The registered practice population

3.4 Any acceptance and exclusion criteria

Included within scope:

9. Adult patients prescribed oral antipsychotic medication who are clinically stable and may remain under Primary Care management while awaiting specialist review.

Exclusions:

10. Children and young people (under 18 years)
11. Patients prescribed antipsychotics under an existing formal shared care agreement, remunerated via the SMS LES
12. Patients prescribed antipsychotics for behavioural and psychological symptoms of dementia (BPSD) – For advice on how to manage these patients, refer to Formulary guidance on Antipsychotics for severe agitation, aggression or psychosis in people with dementia
13. Injectable antipsychotics
14. Non mental health conditions e.g. palliative care, Huntington's disease, intractable hiccups etc.
15. Patients who were initiated by a GP independently without this being requested by a specialist

3.5 Interdependencies with other services

Role of GP

16. The prescriber of the medication legally assumes clinical responsibility for the drug and the consequences of its use. The prescriber should:
17. keep informed about the medicines that are prescribed for the patient
18. be able to recognise serious and frequently occurring adverse side effects
19. make sure appropriate clinical monitoring arrangements are in place and that the patient and healthcare professionals involved understand them
20. keep up to date with relevant guidance on the use of the medicines and on the management of the patient's condition
21. to seek advice from specialist should a patient's condition worsen

Role of Secondary Care Mental Health Providers:

22. support while waiting via helpline and escalation processes
23. triage to prioritise patients
24. to share useful info with ICB for planning purposes
25. to inform referring GP what priority has been allocated to the patient and to provide estimated wait time

Role of VCSE (where available and may include):

26. Substance misuse support

27. Well-being group support

4. Applicable Service Standards

NICE guidelines:

- [CG31](#): Obsessive-compulsive disorder and body dysmorphic disorder: treatment
- [CG142](#): Autism spectrum disorder in adults: diagnosis and management
- [CG178](#): Psychosis and schizophrenia in adults: prevention and management
- [CG185](#): Bipolar disorder: assessment and management
- [NG11](#): Challenging behaviour and learning disabilities: prevention and interventions for people with learning disabilities whose behaviour challenges
- [NG54](#): Mental health problems in people with learning disabilities: prevention, assessment and management
- [NG116](#): Post-traumatic stress disorder
- [NG222](#): Depression in adults: treatment and management

5. Key Service Outcomes

- Patients will be referred to Secondary Care for specialist assessment, including a review of the appropriateness of ongoing antipsychotic prescribing and overall treatment plans.
- Patients will receive care within a Primary Care setting while awaiting specialist review, with a focus on ensuring that their physical health is appropriately monitored and managed during this period.
- General Practitioners (GPs) will have timely access to support and clinical advice from Mental Health Providers, including Livewell Southwest and Devon Partnership NHS Trust (DPT), to aid in the safe and effective management of patients.

6. Applicable Quality Requirements

Applicable Quality Requirements

Not applicable

7. Financial Requirements

Claims should be made at 1 claim per patient per month, regardless of how many antipsychotic drugs are being prescribed. This will include patients on antipsychotics that are not included within the SMS LES and antipsychotics that are within the SMS LES, but have been assessed as tier 1.

A practice cannot claim for a patient who is already being claimed via the SMS LES.

For patients not being claimed under the SMS LES, payments will be made under the Oral antipsychotics in Primary Care - interim care pathway LES in line with tier 2 of the SMS LES as monthly payments.

Tier 2 is funded at £105.94.

For 2026/27, practices can claim retrospectively, back dated to the 1st April 2026 where patients were eligible at that date.

From 1st April 2027,

- a) patients transitioned to shared care will receive ongoing payments via the SMS LES
- b) Patients reviewed by Secondary Care and who remain with Secondary Care (e.g. outside of shared care) cannot be claimed
- c) Patients who remain on the 'Oral antipsychotics in Primary Care - interim care pathway can continue to be claimed monthly via this LES and thereafter on a 12 months rolling period

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

8. Information Requirements

The provider shall submit every month the number of patients to whom the service is provided.

9. Location of Provider Premises

The provider premises.